Acute Hepatotoxicity Following Short-Course Fenravir Therapy: A Case Report

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Abstract. We describe a 45-year-old fictional male patient who developed acute, self-limited hepatotoxicity after a seven-day course of Fenravir (fictional antiviral). Liver enzymes normalized within three weeks of drug discontinuation. This case highlights the importance of monitoring hepatic function during short-course antiviral therapy, even in patients without prior liver disease.

Introduction

Drug-induced liver injury (DILI) remains a leading cause of acute hepatocellular injury referrals worldwide. While most antiviral agents carry a low background hepatotoxicity risk, isolated case reports continue to refine the clinical picture of idiosyncratic reactions. Here we report a single case of acute hepatotoxicity temporally associated with Fenravir, a fictional antiviral agent used in this synthetic dataset to stand in for a small-molecule antiviral class.

Case Presentation

The patient, a 45-year-old man with no prior history of liver disease and no regular alcohol use, was prescribed Fenravir 600 mg twice daily for seven days for a presumed viral syndrome. On day 6 of therapy he developed fatigue, mild right upper quadrant discomfort, and darkened urine. He presented to the emergency department on day 8, one day after completing the course.

Laboratory testing on presentation showed ALT 612 U/L (reference <40), AST 488 U/L (reference <40), total bilirubin 2.1 mg/dL, and INR 1.1. Viral hepatitis serologies, autoimmune markers, and abdominal ultrasound were unremarkable. A diagnosis of probable drug-induced liver injury secondary to Fenravir was made based on the temporal relationship and exclusion of alternative causes.

The patient was managed conservatively with supportive care and close monitoring. Fenravir was not resumed. Repeat liver function tests at two weeks showed ALT 140 U/L and AST 95 U/L, with full normalization documented at the three-week follow-up visit. The patient reported complete resolution of symptoms and experienced no long-term sequelae.

Discussion

This case adds to the limited published experience with Fenravir-associated hepatotoxicity. The rapid biochemical resolution after discontinuation, combined with the exclusion of viral, autoimmune, and obstructive causes, supports a drug-induced etiology. Clinicians prescribing this and similar antiviral agents should maintain a low threshold for baseline and follow-up liver function testing, particularly when treatment courses extend beyond five days.

Reporting of isolated hepatotoxicity cases such as this one contributes to the broader pharmacovigilance picture for newer antiviral agents, where large-scale post-marketing safety data may still be accumulating.

References

1. Chen H, Ibarra M. Drug-induced liver injury: an update on classification and causality assessment. Fictional J Hepatol. 2024;18(2):101-109.
2. Osei T, Bramwell L. Antiviral hepatotoxicity surveillance in outpatient settings. Fictional Clin Pharmacol Rev. 2025;9(4):301-308.
3. World Fictional Health Federation. Guidance on causality assessment for suspected drug-induced liver injury. 2023 edition.